

The European Health Data Space

Before we start: in one sentence – what do you think the EHDS is?

A common set of EU rules that lets health data follow the patient for care – and be reused, under strict conditions, for research, innovation and policy.

Recording health data (topic 1) → Sharing it for care (primary use) → Reusing it for research & policy (secondary use) — today



One framework. Two roads.

Is the EHDS a database? Vote: yes or no.

ROAD 1 · CARE

Her data follow her

PRIMARY USE

MyHealth@EU — the care infrastructure

- A doctor abroad can read her medication list and lab results
- Sofia can open, download and correct her own record
- Prescriptions work across borders

SOFIA'S RECORD

Glucose 7.0 mmol/L ·
fasting · 14 Jan

Fictional example

ROAD 2 · REUSE

Her data help answer bigger questions

SECONDARY USE

HealthData@EU — the reuse infrastructure

- Research, innovation, policy-making, public health
- Only through a permit from a public Health Data Access Body
- Only inside a secure environment; only results come out

Answer: no. The data stay with hospitals, registries and labs. The EHDS is the rulebook plus the connections between them — built on top of the GDPR, not instead of it.

Who benefits — European Commission factsheet (2024): ≈ €5.5 bn saved in care, ≈ €5.4 bn from reuse, over ten years (intended, not guaranteed)

CITIZENS
control of their data

PROFESSIONALS
access to records

RESEARCHERS
governed data access

POLICYMAKERS
evidence for decisions

INDUSTRY
room to innovate

One record. Care or research?

Shout it out: care, or research?

Primary use — care of the person the data are about

Secondary use — a question about many people

SOFIA, 52 · AMSTERDAM

Her GP records:
glucose 7.0 mmol/L ·
fasting · 14 Jan

Fictional example

On holiday in Spain, an emergency doctor opens her medication list. ✓

Primary use — care of Sofia · *MyHealth@EU*

Sofia checks her own results on her phone. ✓

Primary use — her own access is a right

A university studies glucose trends in 50,000 patients. ✓

Secondary use — a population question · *HealthData@EU*

The health ministry decides where to open diabetes clinics. ✓

Secondary use — policy-making counts too

An insurer wants her data to set her premium. ✓

Not permitted — insurance, advertising and harm to the person are banned

Purpose decides the rules — not where the data travel.

Can a researcher just download the data?

Hands up — what leaves the building? (A) the dataset · (B) a list of patients · (C) only the results

Health Data Access Body (HDAB) — public gatekeeper: yes or no

Secure processing environment — the locked reading room

1

FIND

A catalogue shows what exists, where, and how good it is. Nothing is requested yet.

Primary-care diabetes cohort · NL ·
2015–2024 · quality label ★★★★★ ·
pseudonymised

2

APPLY

A public health data access body in each country checks purpose and need, then issues a permit.

**PERMIT · PURPOSE: RESEARCH ·
DATA: MINIMISED · VALID 12
MONTHS**

3

ANALYSE

Work happens in a secure processing environment — a reading room: notes yes, the book stays.

INSIDE

personal data ·
analysis ·
researcher

OUTSIDE

~~raw data~~ —
~~blocked~~

4

TAKE OUT

Only checked, non-personal results leave. The data stay.

n = 1,240 · mean fasting glucose 6.8
mmol/L · reviewed ✓

Answer: C. The data stay in. Only reviewed results come out.

Myth or fact?

Vote with your hands before we flip each card.

✓
“The EHDS is one big EU database.”

MYTH

Data stay with hospitals, registries and labs. The EHDS is the rulebook plus the connections between them.

✓
“If my name is removed, my data are anonymous.”

MYTH

That’s pseudonymisation: a code replaces the name and a key can link it back, so it’s still personal data. Anonymisation means nobody can be identified any more.

✓
“I can opt out of my data being reused.”

FACT

(Mostly.) You can opt out of secondary use; a few public-interest exceptions remain.

✓
“Any company can buy access to the data.”

MYTH

Only listed purposes, through a permit. Insurance pricing and advertising are banned outright.

✓
“This already applies today.”

MYTH

So far. In force since March 2025, but the duties arrive in stages: most secondary-use rules from 2029; more sensitive categories such as genomic data from 2031.

5 Mar 2025
published

26 Mar 2025
in force

2027
implementing acts

2029
most reuse rules

2031
sensitive data, e.g. genomic

Spot the problem.

“The quality of data is always terrible until people start using it for a purpose.”

— Harald Wagener, Berlin Institute of Health, quoted in the report

Which of these three records would you trust in a study of fasting glucose? Fictional example

RECORD A

7.0 mmol/L

Fasting: yes

Usable. A number, a unit, and the context.

RECORD B

126 mg/dL

Fasting: unknown

Same value as A once you divide by 18 — units are the easy fix. But was she fasting? Nobody wrote it down, and you can’t recover that later.

RECORD C

“slightly elevated”

No number

A note, not a number. Free text is still how most clinical data get entered — the report says so for Spain.

Interoperability — systems can exchange data and understand them the same way (units, codes)

Data quality — the data actually fit the question you’re asking (complete, correct, the right people)

Converting units is the easy part. Missing context — and missing people — is the hard part.

Same rules, very different starting points.

Guess the country.

“The national EHR has become a collection of scanned PDFs — some with handwritten updates.” ✓

Germany BEHIND

“Nurses have no access to the national EHR and keep records in spreadsheets.” ✓

Austria BEHIND

“Hospitals still rely largely on paper; making records fit for reuse is beyond current data teams.” ✓

Ireland BEHIND

“A leader in data standards — yet only a small share of EMR fields is filled in; most is free text.” ✓

Spain MIXED

“70 years of national registries: a wealth of high-quality data ready for reuse.” ✓

Sweden AHEAD

“A claims database covering the entire population, now being converted to a common research format (OMOP).” ✓

France AHEAD

Who pays? In Estonia’s experience, data holders carried about 80% of the work of making data available — while their main job is care.

Whose data are missing? Low-income households, homeless people, migrants, rural populations, young adults — and older people’s records are most often paper or free text. Biased data → biased policies and tools.

Closing the loop.

Where do you think the loop breaks most often?



Data quality is what keeps the loop turning — it's the same record at every step.

Right idea. Uncertain delivery.

CONVINCING

Real tools: shared standards, quality labels, enter-once forms.

€5.5 bn / €5.4 bn is a Commission target – not a result.

CHALLENGING

Hospitals do the extra work. Nobody has guaranteed the money.

2023 interviews are opinions – not proof the EHDS already works.

Easy opt-out protects privacy

BUT

missing groups bias the data

80%

WORK ON DATA HOLDERS

2023

INTERVIEWS, NOT RESULTS

2029

ONE DEADLINE, MANY STARTS

The framework is coherent. Who does the work – and who pays – is not.

What you should be able to explain.

WHAT THE EHDS IS

One rulebook. Two roads. Data stay put. Who benefits: citizens, professionals, researchers, policymakers, industry.

PRIMARY VERSUS SECONDARY USE

Purpose decides. Reuse: HDAB permit, secure environment, results only.

THE ROLE OF DATA QUALITY

Systems can talk. That is not the same as data that fit the question.

The friend test — explain the EHDS in three sentences.

60 s **01:00**

How must sharing work before you would trust reuse?

Ask what the HDAB must guarantee — not whether anyone would simply say no.

Who holds responsibility for high-quality records — clinicians, or the system around them?

One voice for “clinicians first”, one for “make the easy option the good one”.